

The court grants final approval of the class action and PAGA action settlement and approves the attorneys' fees, costs, and the service award.

The court retains jurisdiction over the parties to enforce the terms of the judgment. (Cal. Rules of Court, rule 3.769, subd. (h).)

The court schedules this matter for a final accounting hearing on **November 17, 2026 at 8:30 a.m. in Department 32**. Plaintiff shall file and serve a final accounting status report by no later than November 3, 2026.

13. S-CV-0052591 Germany, Steven E v. Bains, Jazmin

Motion for Summary Judgment

Defendants Sutter Valley Hospitals dba Sutter Roseville Medical Center (erroneously sued as Sutter Roseville Medical Center Foundation) and Sutter Health (collectively, "SRMC") move the court for summary judgment or, in the alternative, summary adjudication of plaintiffs' complaint pursuant to Code of Civil procedure section 437c. Plaintiffs oppose the motion.

Preliminary Matters

Defendants object by way of reply brief to the declaration of plaintiff Steven E. Germany filed together with the opposition brief on May 27, 2026. However, as defendants' objection does not comply with Rule 3.1354 of the California Rules of Court, the objection is overruled.

Plaintiffs submitted a supplemental declaration of plaintiff Steven E. Germany on June 5, 2026—after the briefing period for any opposition had already expired. (Code Civ. Proc., § 437c, subd. (b)(2).) Defendants object to the court considering the supplemental declaration. The court sustains the objection and does not consider plaintiffs' supplemental declaration.

Ruling on the Motion

A party is entitled to bring a motion for summary judgment where there are no triable issues of fact and the moving party is entitled to judgment as a matter of law. (Code Civ. Proc., § 437c, subd. (c).) The defendant bears the initial burden of establishing that one or more elements of the cause of action cannot be established or that there is a complete defense to the cause of action. (*Id.* subd. (p)(2).) Only when this initial burden is met does the burden shift to the opposing party to show a triable issue of material fact. (*Ibid.*) A party may move for summary adjudication as to one or more causes of action if the party contends the cause of action has no merit. (*Id.* subd. (f)(1).) A party may move for summary adjudication as an alternative to summary judgment and shall proceed in all procedural respects as a motion for summary judgment. (*Id.* subd. (f)(2).) In reviewing a

motion for summary judgment, the court must view the supporting evidence, and inferences reasonably drawn from such evidence, in the light most favorable to the opposing party. (*Aguilar v. Atlantic Richfield Co.* (2001) 25 Cal.4th 826, 843.) The court reviews the motion with these principles in mind.

Defendants present evidence that on January 18, 2023, Wren Germany (“Mr. Germany”) was presented to the Sutter Roseville Medical Center Emergency Department complaining of weakness and shortness of breath. (SSUMF No. 2.) Dr. Arthur Jey evaluated Mr. Germany, a 96-year-old man with a history of hypertension, chronic kidney disease stage 3, chronic atrial fibrillation on Coumadin, diabetes mellitus type 2, congestive heart failure, and osteoarthritis. (SSUMF Nos. 3, 6.) Dr. Jey ordered a CT with contrast, which showed a possible left small pulmonary embolism, and admitted Mr. Germany. (SSUMF Nos. 4, 54.) Dr. Jazmin Bains observed the first CT was unclear as to pulmonary embolism and on January 20, 2023, ordered a second CT with contrast, which showed possible pulmonary embolism. (SSUMF Nos. 16, 26, 55–56.) Mr. Germany thereafter suffered CT contrast-induced nephropathy. (SSUMF Nos. 26, 31.)

Throughout Mr. Germany’s admission (from January 19 through 26, 2023), his vital signs were documented several times throughout each day; medication was administered numerous times throughout each day; Mr. Germany was described as awake, alert, and generally oriented; and his lab results included rising levels of creatinine. (SSUMF Nos. 7–10, 12–15, 17–20, 22–25, 27–30, 33–36, 39–42, 47–50.) Nursing staff and the attending physicians noted Mr. Germany was experiencing no pain generally or no chest pain specifically on January 19, 20, 21, 22, 23, and 26, 2023, although discomfort was noted on January 19, 2023. (SSUMF Nos. 7–8, 11, 13, 18, 21–23, 27–28, 47–48.) However, on January 24, 2023, nursing notes refer to morning and nighttime pain relieved by medication and in the early hours on January 25, 2023, Fentanyl was ordered to address Mr. Germany’s pain and nursing notes indicate pain is improving after medication. (SSUMF Nos. 34, 40, 44.)

On January 20, 2023, the attending physician noted CT imaging concerning for possible pulmonary embolism and that Mr. Germany continued to experience shortness of breath. (SSUMF No. 16.) The next day, the attending physician noted imaging that showed distal pulmonary embolism and that Mr. Germany was feeling well but tired. (SSUMF No. 21.) On January 22, 2023, the attending physician noted a new pulmonary embolism per chest imaging and possible CT contrast-induced nephropathy and spoke with a nephrologist to create a treatment plan including more fluids and to administer Heparin. (SSUMF No. 26.) On January 23, 2023, the attending physician noted acute kidney injury due to contrast nephropathy and that Mr. Germany remained on the Heparin drip through January 25, 2023. (SSUMF Nos. 31, 44.) On January 24, 2023, a nephrologist indicated that Mr. Germany’s breathing had improved and that the thoracentesis had removed 1.1 liter of fluid; the attending physician noted continued worsening of renal function. (SSUMF Nos. 37–38.) On January 26, 2023, Mr. Germany was discharged home with plans for hospice. (SSUMF No. 53.)

Mr. Germany consented to the advisement that “all physicians . . . are not employees or agents of the Hospital” and all physicians involving care “are not employees or agents of the Hospital” orally on January 18, 2023, October 9, 2022, and April 14, 2022; and in writing on August 12, 2022, January 6, 2018, and July 7, 2015. (SSUMF Nos. 57–59.) Mr. Germany consented to the advisement that “all physicians . . . are independent contractors and are not employees or agents of the Hospital” on December 19, 2011, June 2, 2011, December 10, 2008, November 19, 2007, August 11, 2006, November 18, 2005, and November 7, 2005. (SSUMF No. 60.)

In their first and second issues, defendants contend plaintiffs’ claims fail because defendants’ nursing staff met the standard of care and that the standard of care does not require defendants to have a policy preventing contrast-induced nephropathy or a policy prohibiting physicians from ordering contrast CT scans. These arguments rely on facts not included in the separate statement. These two arguments rely primarily on the declarations of Nurse Stephanie Iseri and Nurse Kathryn Biasotti, neither of whom are mentioned in the separate statement, nor are their expert opinions. A separate statement shall “set[] forth plainly and concisely all material facts that the moving party contends are undisputed. . . . The failure to comply with this requirement of a separate statement may in the court’s discretion constitute a sufficient ground for denying the motion.” (Code Civ. Proc., § 437c, subd. (b)(1).) While the court is not precluded from considering evidence outside of the separate statement, it is under no obligation to do so. (See *San Diego Watercrafts, Inc. v. Wells Fargo Bank, N.A.* (2002) 102 Cal.App.4th 308, 315 [holding the question of whether to consider evidence not referenced in a separate statement rests with the sound discretion of the trial court].) The court declines to consider evidence outside of the parties’ respective separate statements for consideration of this motion. As to the first two issues, defendants fall short of their initial burden. Even if defendants had met their initial burden on their first and second issues, the outcome would still be the same because plaintiffs demonstrate one or more triable issues of material fact. (AMF Nos. 21, 22, 24, 25, 27–39.)

In their third and final issue, defendants contend they owe no vicarious duty to plaintiffs for the alleged malpractice of physicians because physicians were not defendants’ employees or agents. While defendants present evidence that Mr. Germany was advised numerous times that the physicians at Sutter Roseville Medical Center were not agents or employees of Sutter Roseville Medical Center (SSUMF Nos. 57–60), defendants submit no evidence that physicians are not in fact their agents or employees. Accordingly, defendants fall short of their initial burden on the third issue as well.

Based on the foregoing, the motion for summary judgment is denied and the motion for summary adjudication is denied.